Financial Pressure Across Medi-Cal Healthcare Plans

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Medi-Cal

Medi-Cal on the Line

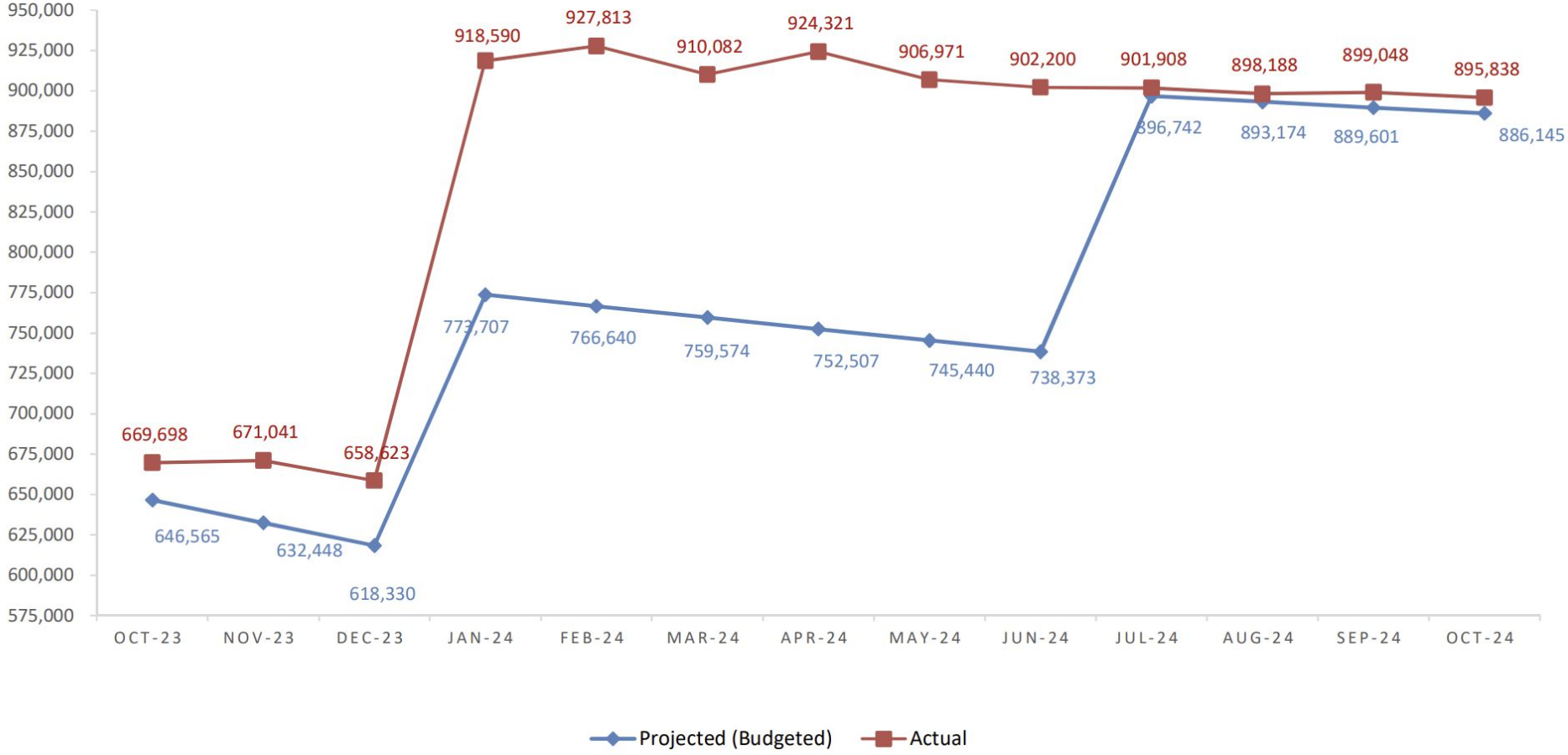
Medi-Cal is **California's version** of the Federal Medicaid program.

Less than a month ago, Gavin Newsom released the revised 2025-26 state budget to address a **projected $12 billion deficit.**

The administration is relying on $6B reduction on Medi-Cal

Medi-Cal risks cuts to the most important programs to California's residents.

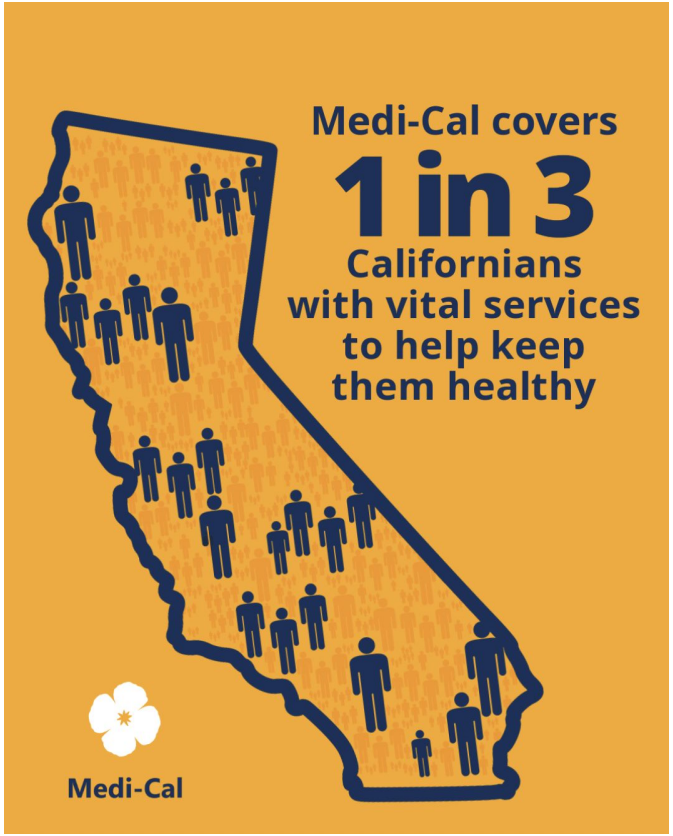
PARTNERSHIP HEALTHPLAN OF CALIFORNIA
ACTUAL V. PROJECTED MEDI-CAL ENROLLMENT
OCT 2023 - OCT 2024



Medi-Cal's Importance

Medi-Cal is the Primary source of health insurance for 13+ million people in California - that's **1 in 3 residents**

- Some counties have over half of their population enrolled in Medi-Cal.
- Nearly **4 out of 10 babies** born in California in 2022 were delivered through Medi-Cal coverage
- **$175+ billion budget**—making it one of the state's largest expenses



Who is Eligible?

- **Low-income adults (ages 19-64)** - Making up to 138% of Federal Poverty Level

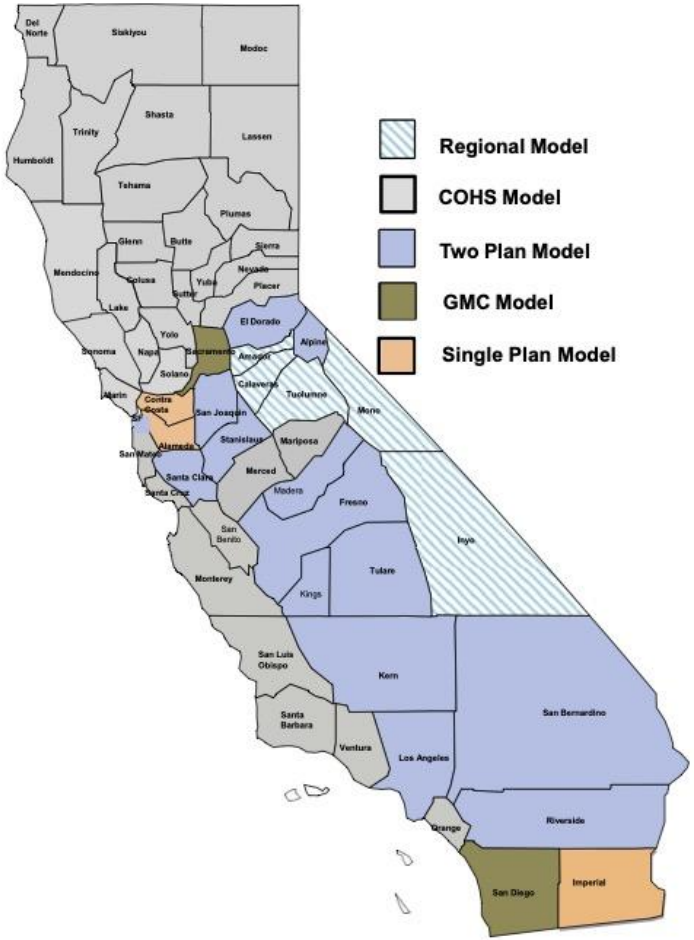
- **California's most vulnerable** - Seniors 65+, people with disabilities, low-income children, pregnant women, and nursing home residents

MEDI-CAL MANAGED CARE MODELS
As of January 1, 2024

Regional Model
Used in rural counties with small populations.
State selects 1–2 commercial plans.

County Organized Health System (COHS)
1 Plan Managed by the local county entity.

Two Plan Model
1 Public plan (Local Initiative)
&
1 commercial plan (usually Anthem or Health Net)



Geographic Managed Care (GMC)
Residents choose from multiple commercial plans

(the government pays the commercial healthcare company a fixed amount from taxes and plan is free for residents)

Single Plan Model
One single Plan

(State contracts with the health plan, not the county)

County Organized Health System Model (COHS)

- **County-run health plan:** All Medi-Cal recipients are mandatorily enrolled in the single COHS plan.
- **Mandatory:** County government runs the health plan instead of private insurance companies.
- **What Counties?:** There are 6 COHS plans that operate in 22 counties (some counties combine under the same plan).

Our Exploration Focus

How does switching to COHS/Single Plan affect expenditure per person and the county enrollment rate?

Data Source for Enrollment

Data Source: Medi-Cal Eligibility Statistics provided by Department of Health Care Services (DHCS)

1: Medi-Cal Managed Care Enrollment Report

We observe number of Enrollment of the county, its Plan Model by month

2: Medi-Cal Certified Eligible Counts, by Age Group and Sex

Monthly eligible counts by County, Age Group, and Sex

We merged the dataset to form a monthly panel and compute the enrollment rate of eligible people per county

Data Source for Expenditure

Data Source: Medi-Cal Local Assistance Estimates provided by DHCS

1: Medi-Cal Reported Member Months and Total Spending

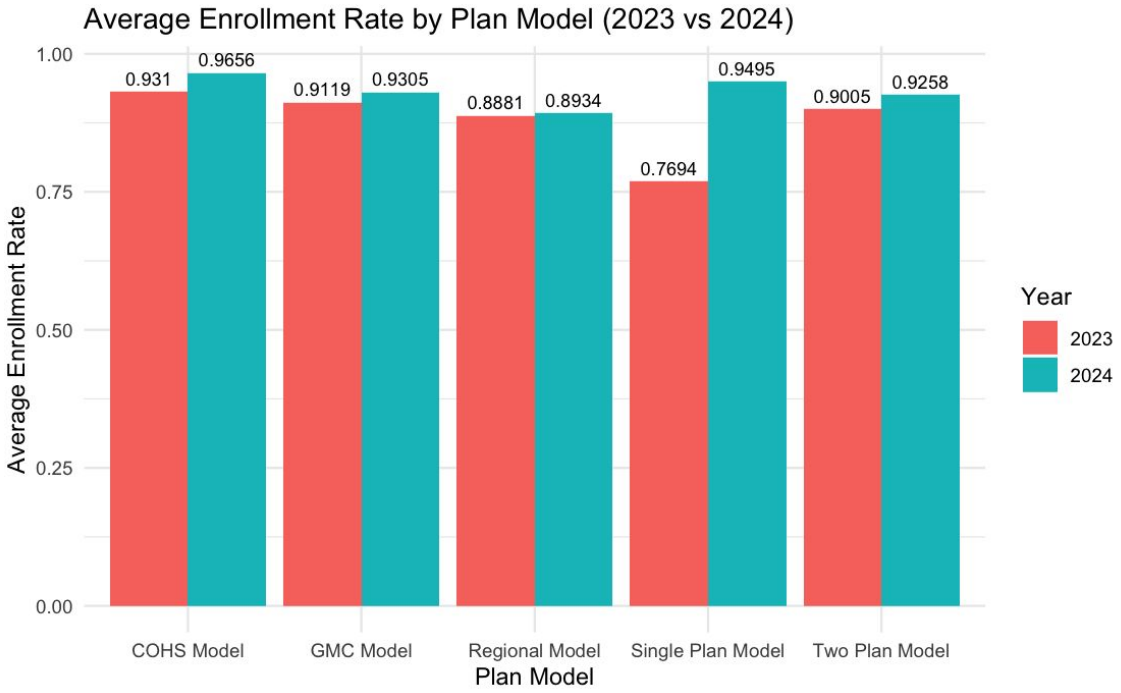
Manually extracted the data from these types of tables in the reports for each year:

25. GMC dollars on an accrual basis are:
(Dollars in Thousands)

FY 2023-24	Member Months	Total
Sacramento	7,110,545	$2,034,384
San Diego	11,623,592	$3,670,647
Total FY 2023-24	18,734,137	$5,705,032
Maternity (events)	18,554	$163,457
Total FY 2023-24 with Maternity		**$5,868,489**

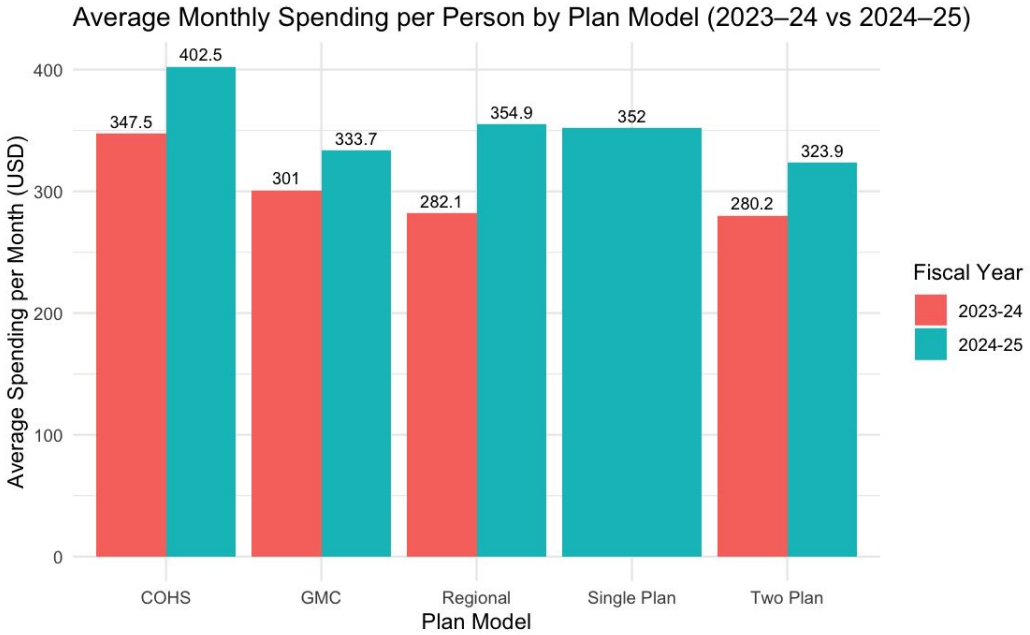
Average Enrollment Rates for 2023 and 2024

Plan Model	Year	Average Enrollment Rate
COHS Model	2023	0.9310
COHS Model	2024	0.9656
GMC Model	2023	0.9119
GMC Model	2024	0.9305
Regional Model	2023	0.8881
Regional Model	2024	0.8934
Single Plan Model	2023	0.7694
Single Plan Model	2024	0.9495
Two Plan Model	2023	0.9005
Two Plan Model	2024	0.9258



Average Monthly Spending for 2023-24 and 2024-25

Plan Model	Fiscal Year	Average Monthly Spending per Person (USD)
COHS	2023-24	347.5
COHS	2024-25	402.5
GMC	2023-24	301.0
GMC	2024-25	333.7
Regional	2023-24	282.1
Regional	2024-25	354.9
Single Plan	2024-25	352.0
Two Plan	2023-24	280.2
Two Plan	2024-25	323.9



How does Managed Care Model affect Medicaid Enrollment Rate

We built a county-level panel data by month from Jan 2010 - June 2022 and July 2023 - Aug 2024, we focus on the mass model switch on Jan 2024 with data window July 2023 to July 2024

County	Managed Care Model	Enrollment	Year Month	Number of Eligible Person	Enrollment Rate of Eligible People
San Francisco	Two Plan Model	230863	2024-06	249612	0.9248874
San Joaquin	Two Plan Model	302848	2024-06	323975	0.9347882
San Luis Obispo	COHS Model	67507	2024-06	68647	0.9833933
San Mateo	COHS Model	160365	2024-06	169428	0.9465083
Santa Barbara	COHS Model	172846	2024-06	178281	0.9695144
Santa Clara	Two Plan Model	436692	2024-06	466171	0.9367635
Santa Cruz	COHS Model	79438	2024-06	82746	0.9600222
Shasta	COHS Model	68176	2024-06	70147	0.9719019
Sierra	COHS Model	840	2024-06	815	1.0306748
Siskiyou	COHS Model	18112	2024-06	18953	0.9556271

Method-Difference in Difference

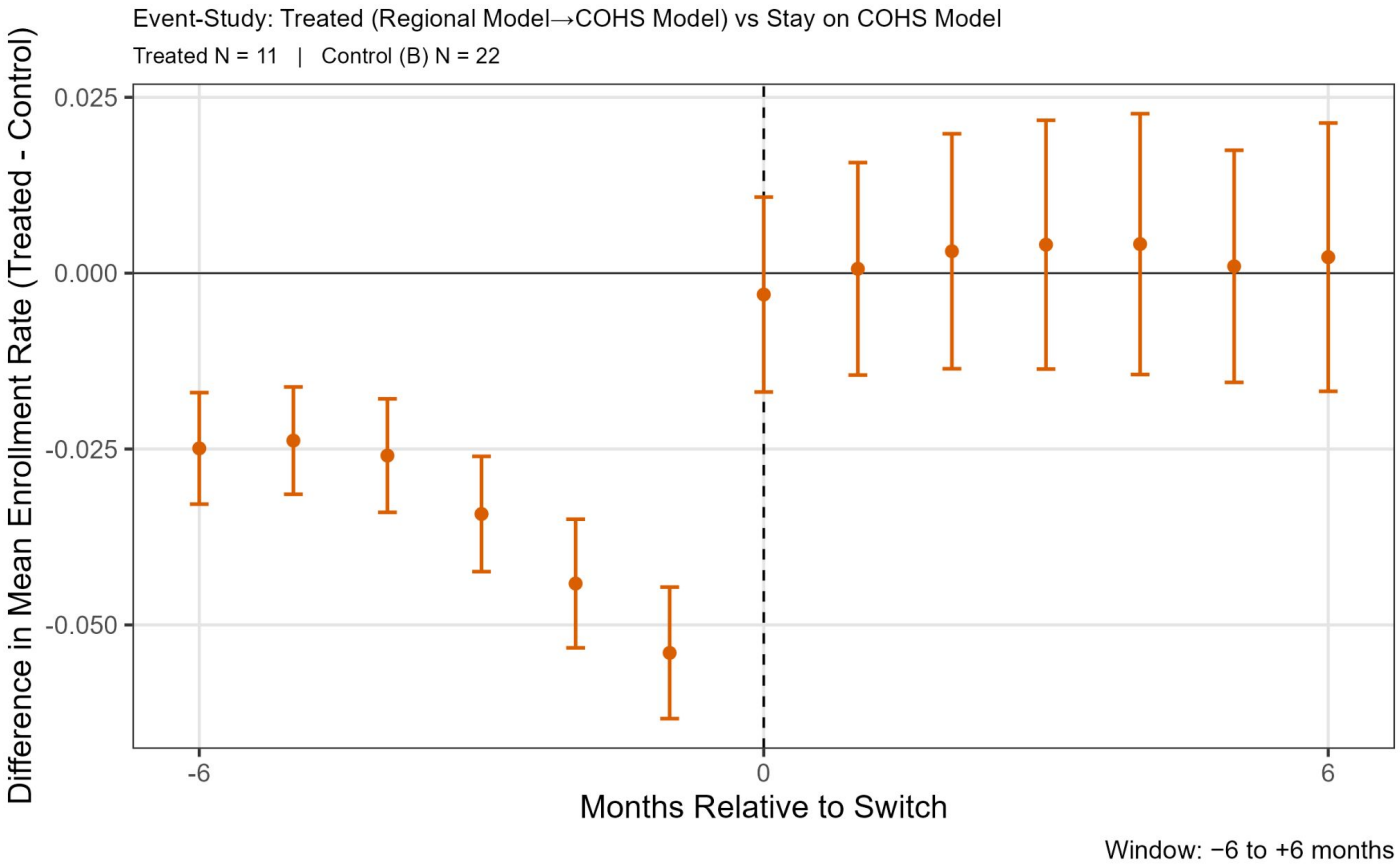
Minimal Spillover: Medi-Cal members can enroll only in Managed Care Plan (MCP) offered in their home county, spillover effect is minimal.

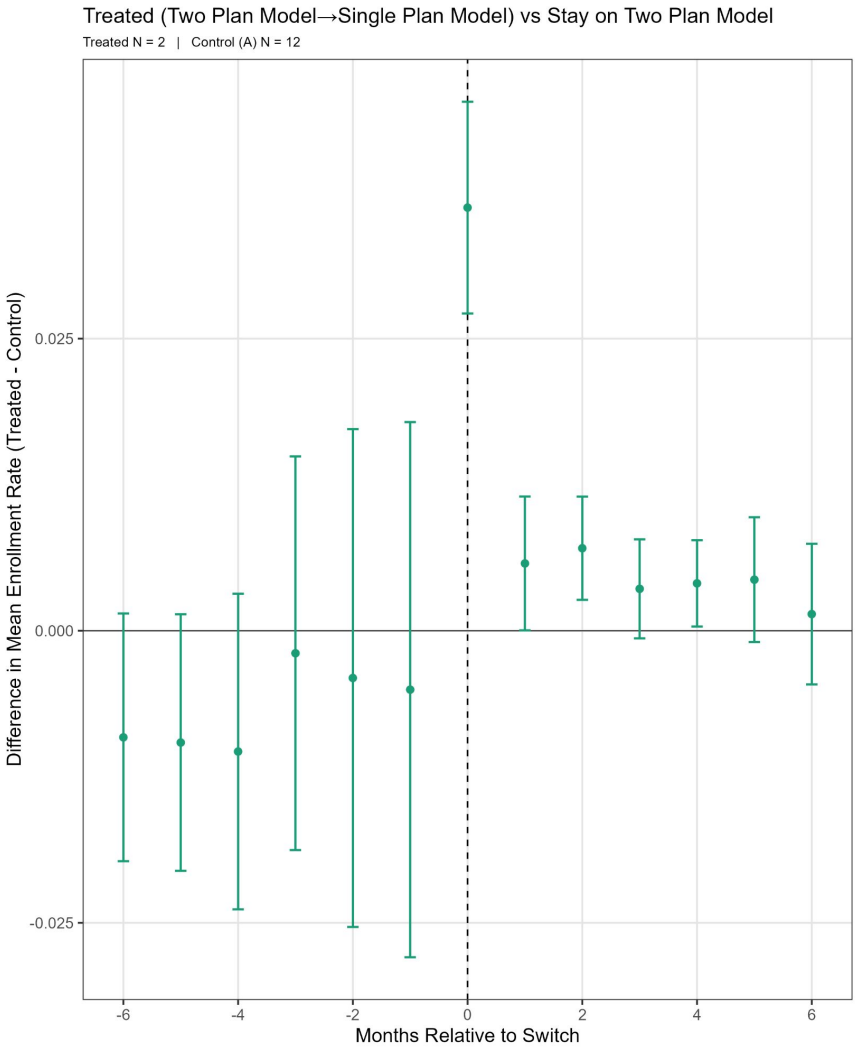
Stable Treatment & Control Groups: Every enrollee joins an MCP within the resident county and we only use 1 year time window, the composition of treated and control counties remains consistent over time.

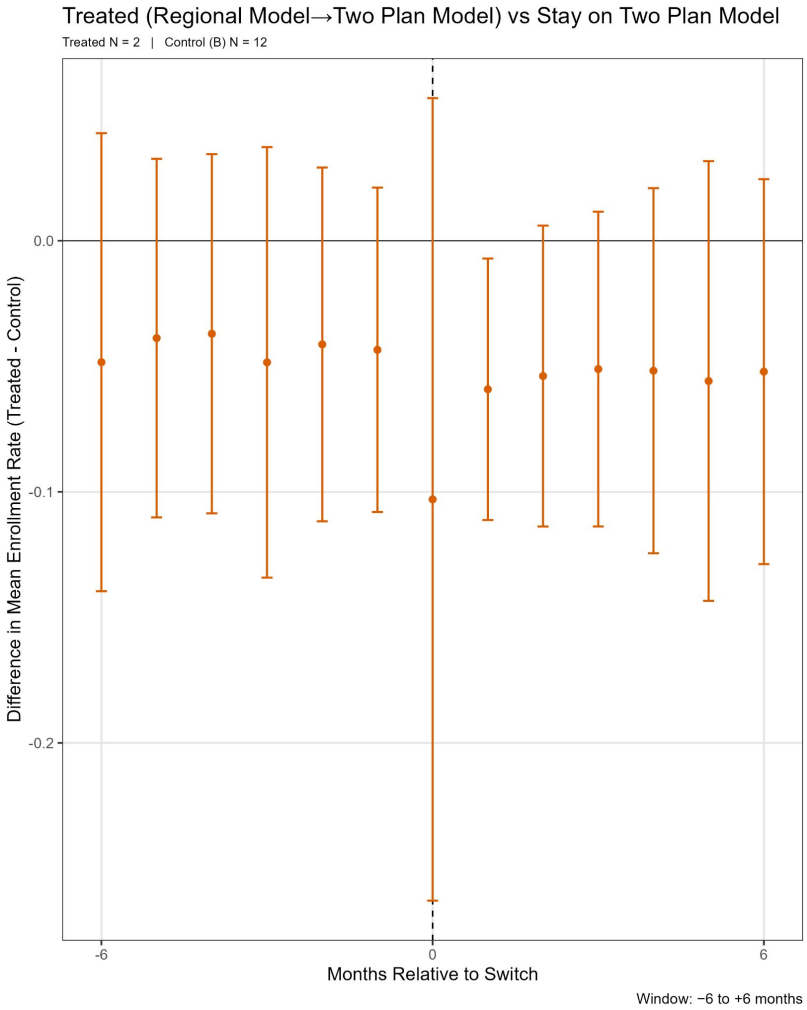
Parallel Trends Confirmed: Pre-policy enrollment rates move in parallel across plan groups, validating the parallel-trend assumption and giving us flexibility in selecting a control group.

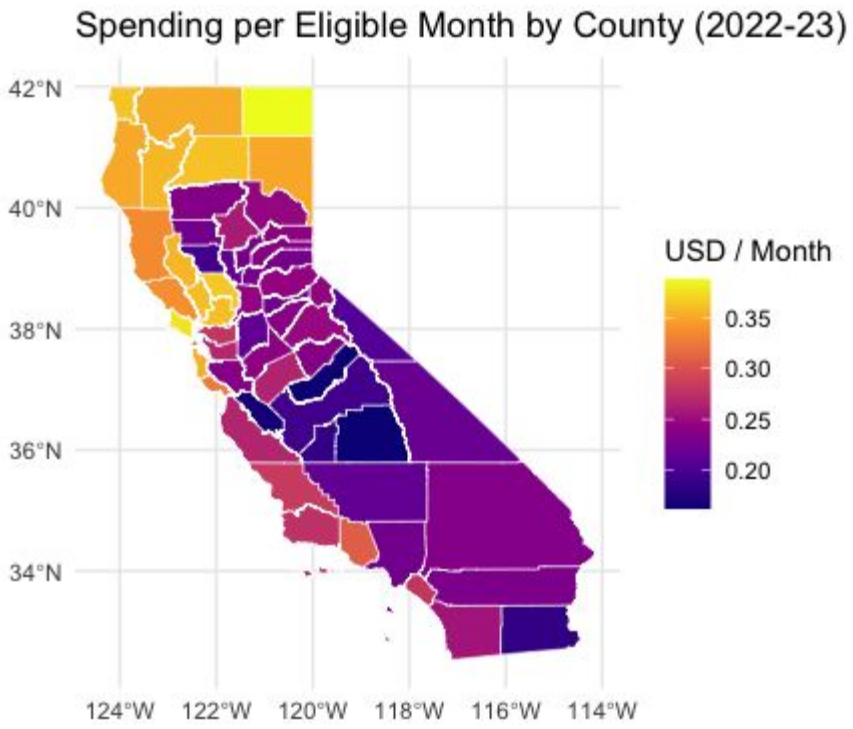
Treatment Definition & Comparison:

- *Treatment:* Counties that transition from **Plan A** to **Plan B**.
- *Control:* Non-switchers remaining in Plan A and Plan B; the plan with more participating counties serves as the control for robustness.
- *Estimation:* Compare mean enrollment-rate changes between switchers (treated) and non-switchers (control).

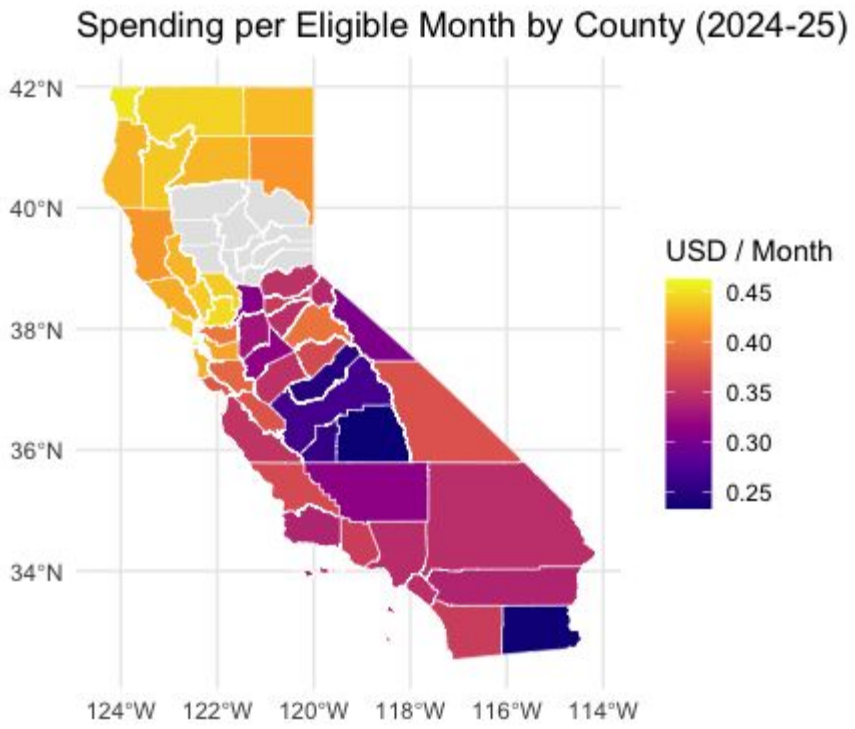

Event-Study: Treated (Regional Model→COHS Model) vs Stay on COHS Model
Treated N = 11 | Control (B) N = 22
Difference in Mean Enrollment Rate (Treated - Control)
0.025
0.000
-0.025
-0.050
-6
0
6
Months Relative to Switch
Window: −6 to +6 months

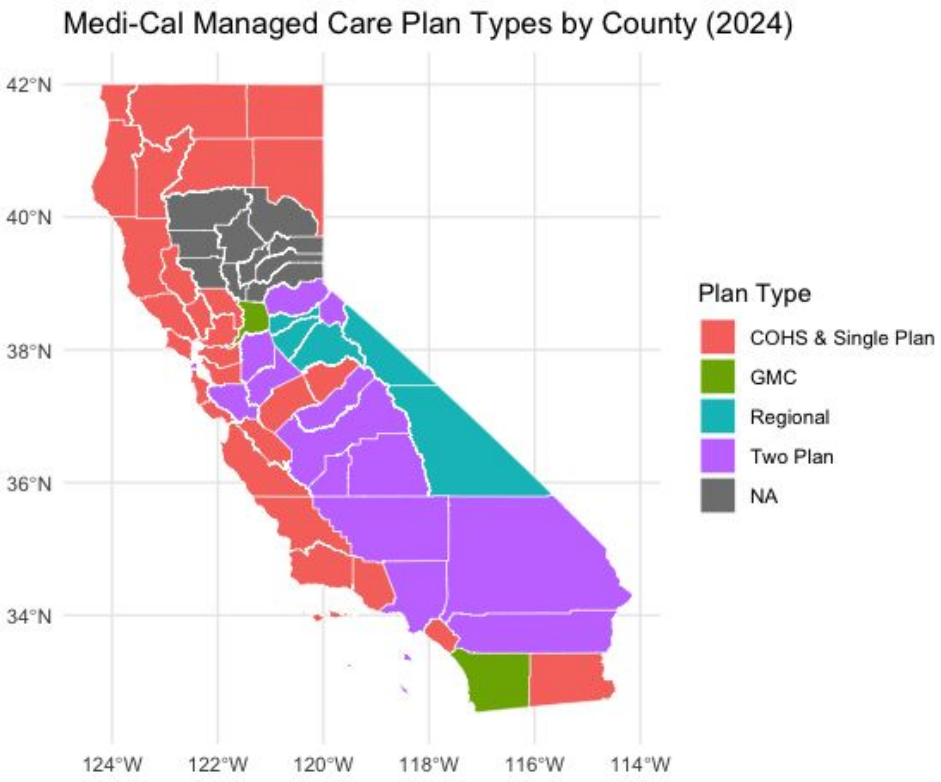

Treated (Two Plan Model→Single Plan Model) vs Stay on Two Plan Model
Treated N = 2 | Control (A) N = 12
Difference in Mean Enrollment Rate (Treated - Control)
0.025
0.000
-0.025
-6
-4
-2
0
2
4
6
Months Relative to Switch


Treated (Regional Model→Two Plan Model) vs Stay on Two Plan Model
Treated N = 2 | Control (B) N = 12
Difference in Mean Enrollment Rate (Treated - Control)
0.0
-0.1
-0.2
-6
0
6
Months Relative to Switch
Window: −6 to +6 months


Spending per Eligible Month by County (2022-23)
42°N
40°N
38°N
36°N
34°N
124°W
122°W
120°W
118°W
116°W
114°W
USD / Month
0.35
0.30
0.25
0.20

California Medi-Cal Plan Types by County before 2024
42°N
40°N
38°N
36°N
34°N
124°W
122°W
120°W
118°W
116°W
114°W
Plan Type
COHS
GMC
Regional
Two Plan


Spending per Eligible Month by County (2024-25)
42°N
40°N
38°N
36°N
34°N
124°W
122°W
120°W
118°W
116°W
114°W
USD / Month
0.45
0.40
0.35
0.30
0.25

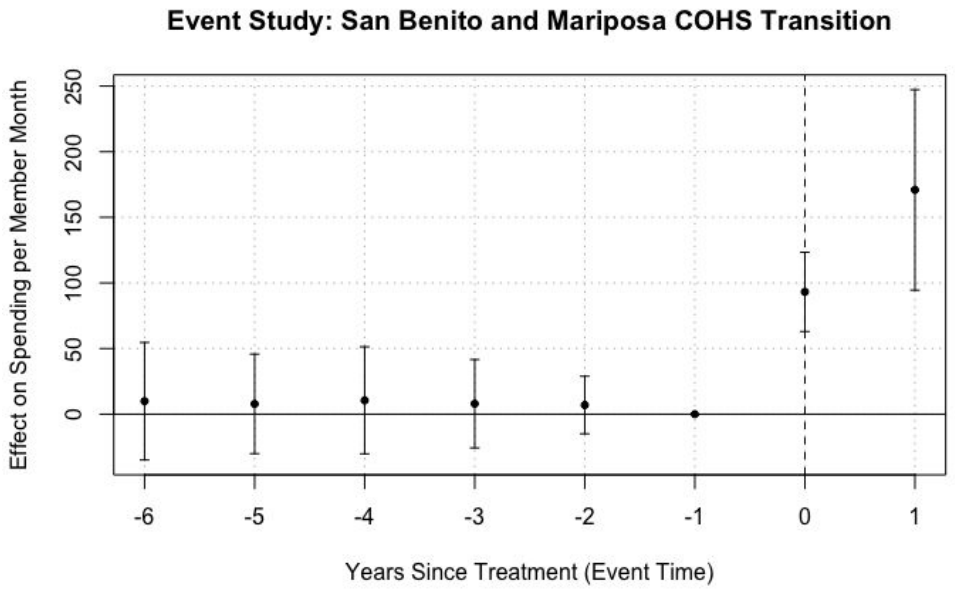

Medi-Cal Managed Care Plan Types by County (2024)
42°N
40°N
38°N
36°N
34°N
124°W
122°W
120°W
118°W
116°W
114°W
Plan Type
COHS & Single Plan
GMC
Regional
Two Plan
NA

$$\underbrace{\overline{\text{Spend}}_{if}}_{\substack{\text{average monthly}\\ \text{spend per eligible person}\\ \text{in county } i \text{ during fiscal year } f}} = \sum_{k \neq -1} \beta_k \underbrace{\left[\mathbf{1}\{\text{EventTime}_{if} = k\} \times \text{Treated}_i\right]}_{\substack{\text{event-time dummy } k\\ \text{for treated counties only}}} + \underbrace{\gamma_i}_{\text{county fixed effect}} + \underbrace{\delta_f}_{\text{fiscal-year fixed effect}} + \underbrace{\varepsilon_{if}}_{\text{error term}} .$$

Term-by-term guide

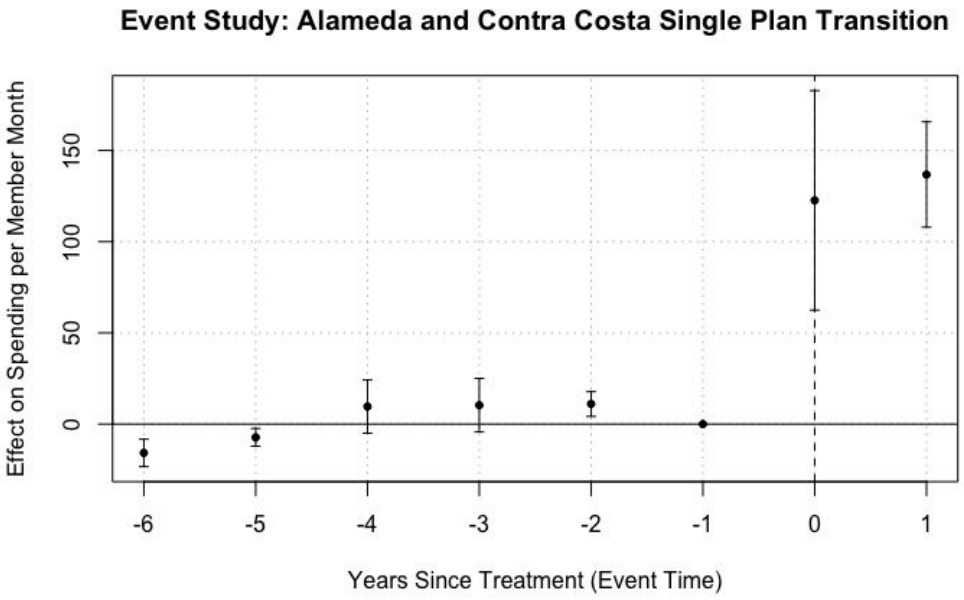
Symbol	Meaning
$\overline{\text{Spend}}_{if}$	**Average monthly spending per eligible person** in county i over fiscal year f.
$\mathbf{1}\{\text{EventTime}_{if} = k\}$	Indicator that fiscal year f is exactly k years before ($k < 0$) or after ($k > 0$) the event for county i.
Treated_i	Equals 1 if county i ever receives the treatment (0 otherwise).
β_k	Event-study coefficient: change in the outcome in relative year k **compared with reference year** $k = -1$.
γ_i	**County fixed effect** absorbing time-invariant differences across counties (baseline cost levels, demographics, etc.).
δ_f	**Fiscal-year fixed effect** capturing shocks common to every county in fiscal year f (e.g., reimbursement changes, nationwide policy shifts tied to the fiscal calendar).
ε_{if}	Idiosyncratic error term; cluster at the county level to correct standard errors for within-county correlation.

Transition from Regional to COHS Model




```
OLS estimation, Dep. Var.: spending_per_member_month
Observations: 58
Fixed-effects: county: 7
Standard-errors: Clustered (county)
                        Estimate Std. Error  t value   Pr(>|t|)
event_time::-6:treated   9.85377   18.30094 0.538430 0.60965845
event_time::-5:treated   7.83534   15.49995 0.505507 0.63123131
event_time::-4:treated  10.55828   16.65615 0.633897 0.54953536
event_time::-3:treated   7.96300   13.75478 0.578926 0.58370252
event_time::-2:treated   6.97924    8.95760 0.779141 0.46550703
event_time::0:treated   93.17078   12.33762 7.551761 0.00027979 ***
event_time::1:treated  170.87813   31.20835 5.475398 0.00155007 **
---
Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1
RMSE: 37.6     Adj. R2: 0.436592
             Within R2: 0.415239
```

Transition from Two Plan to Single Plan Model




```
OLS estimation, Dep. Var.: spending_per_member_month
Observations: 112
Fixed-effects: county: 14
Standard-errors: Clustered (county)
                        Estimate Std. Error  t value   Pr(>|t|)
event_time::-6:treated -15.82431    3.47925 -4.54820 5.4694e-04 ***
event_time::-5:treated  -7.27337    2.26178 -3.21577 6.7592e-03 **
event_time::-4:treated   9.61273    6.75080  1.42394 1.7802e-01
event_time::-3:treated  10.35539    6.79267  1.52449 1.5134e-01
event_time::-2:treated  10.99778    3.14026  3.50218 3.8977e-03 **
event_time::0:treated  122.63858   27.84866  4.40375 7.1272e-04 ***
event_time::1:treated  136.83380   13.37661 10.22933 1.3822e-07 ***
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Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1
RMSE: 33.0     Adj. R2: 0.606875
             Within R2: 0.293879
```

Findings

Switching to the COHS managed care model increased enrollment yet increased costs.

Switching to the Single Plan managed care model did not change enrollments yet also increased costs.

Because counties switched to COHS/Single Plan Models, the state had to bear the extra costs.

Conclusion

The state policymakers should prioritize communicating this information with the counties to limit health cost-overruns while protecting community health.